

MedCorp General Hospital

Department of Pathology & Laboratory Medicine

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NABL Accredited Laboratory | Reg No: MH-LAB-2019-0042

COMPLETE BLOOD COUNT (CBC) REPORT

Patient Name:	Rajesh Kumar	Report No:	LAB-2024-08847
Age / Gender:	45 Years / Male	Sample Date:	March 3, 2024 07:30 AM
Patient ID:	PT-2024-1142	Report Date:	March 3, 2024 11:15 AM
Ward / Room:	Ward 4 / Room 12	Referred By:	Dr. Sharma
Blood Group:	B Positive (B+)	Sample Type:	Venous Blood (EDTA)

TEST NAME	RESULT	UNIT	REFERENCE RANGE	STATUS
Haemoglobin (Hb)	9.2	g/dL	13.8 - 17.2	LOW
Red Blood Cells (RBC)	3.8	million/uL	4.5 - 5.9	LOW
White Blood Cells (WBC)	11,400	/uL	4,000 - 11,000	HIGH
Platelets (PLT)	145,000	/uL	150,000 - 400,000	LOW
Haematocrit (HCT)	31.2	%	41.0 - 53.0	LOW
Mean Corpuscular Volume (MCV)	72.4	fL	80.0 - 100.0	LOW
MCH	24.2	pg	27.0 - 33.0	LOW
MCHC	29.5	g/dL	31.5 - 36.0	LOW
Neutrophils	78	%	40 - 70	HIGH
Lymphocytes	14	%	20 - 40	LOW
Monocytes	6	%	2 - 10	NORMAL
Eosinophils	1.5	%	1 - 6	NORMAL
Basophils	0.5	%	0 - 1	NORMAL
ESR (Erythrocyte Sedimentation Rate)	42	mm/hr	0 - 15	HIGH
Blood Glucose (Fasting)	187	mg/dL	70 - 100	HIGH
HbA1c (Glycated Haemoglobin)	8.4	%	Below 5.7	HIGH
Serum Iron	38	ug/dL	60 - 170	LOW
Total Iron Binding Capacity (TIBC)	480	ug/dL	250 - 370	HIGH
Ferritin	6.2	ng/mL	12 - 300	LOW
Vitamin B12	142	pg/mL	200 - 900	LOW
Creatinine	1.3	mg/dL	0.7 - 1.2	HIGH
Urea (BUN)	28	mg/dL	7 - 25	HIGH

CRITICAL FLAGS & CLINICAL NOTES

1. Haemoglobin critically low at 9.2 g/dL — indicative of moderate-to-severe Iron Deficiency Anaemia.
2. Elevated WBC count (11,400) with neutrophilia (78%) — possible active infection or inflammatory response. Recommend blood culture.
3. Fasting glucose 187 mg/dL with HbA1c 8.4% — poorly controlled Type 2 Diabetes. Immediate medication review recommended.
4. Low Ferritin (6.2) and high TIBC (480) confirm iron deficiency. IV iron supplementation may be required.
5. Low Vitamin B12 (142 pg/mL) — B12 supplementation required immediately.
6. Mildly elevated Creatinine and Urea — monitor renal function. Repeat KFT in 2 weeks.
7. Microcytic hypochromic anaemia pattern (low MCV, MCH, MCHC) consistent with iron deficiency.

Dr. Anita Verma MD Pathology MedCorp General Hospital
Reg: MH-DOC-4421

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